Patient Name: Clara Vaughn
DOB: 12/09/1983
Reason for referral: average risk
Patient phone: 530-555-2019
Primary insurance: Humana

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WakeMed Physician Practices
wakemedphysicians.com

# Request for Referral to Gastroenterology

Referral Status *(circle one)*: **STAT** **Urgent** **Routine**

Date:_____/_____/_____

Referring Provider: _____ Practice Name: _____

Practice phone: _____ Practice fax: _____

Patient First Name: _____ Patient Last Name: _____

Date of Birth: _____/_____/_____ Copy of insurance card attached (required)

Best contact phone: _____ Medical Insurance ID #: _____

Patient Address: _____

**Reason for referral:** office consult screening colonoscopy (without office consult)

Direct scheduling: Please specify procedure requested (referring MD states no office consult needed before procedure):

Procedure requested: _____

Specific question to be addressed in procedure: _____

**Diagnosis code** (symptoms): _____

_____

Specific Question to be addressed in office consult: _____

_____

☐ Please fax recent/last note, medication list, pertinent labs, copy on insurance card, any GI procedure notes

Refer to: **Next Available** provider or **specific provider** *(circle one provider below)*:

**3024 New Bern Ave, Raleigh or 10010 Falls of Neuse Rd, Raleigh**

Ph: 919-350-5318 Fax: 919-350-7093

Dr Riaz Chowdhury
Dr Adeyemi Lawal
Dr Juan Herrera
Dr Shannon Scholl
Dr Luis Vazquez-Montesino

**110 Kildaire Park Drive, Cary**

Ph: 919-235-6440 Fax: 919-235-1389

Dr Dina Ahmad
Dr Mohammad Esmadi
Dr Paul Oh
Dr Sara Ali (after 6/2025)

Please note this form must be completed in its entirety to be processed by WakeMed Gastroenterology.

**ClinLab**
Clinical Testing Laboratory, Inc.

Clinical Testing Lab of California
55492 Spring Street, Suite 201
Vienna, CA 94078-9932
Phone: 510-555-1212

| Specimen Number | Patient ID | Control Number | Account Number | Account Phone Number | Route |
|---|---|---|---|---|---|
| | | | | | |

| Patient Last Name | Account Address |
|---|---|
| | |

| Patient First Name | Patient Middle Name |
|---|---|
| Jeff | |

| Patient SS# | Patient Phone | Total Volume |
|---|---|---|
| | | |

| Age (Y/M/D) | Date of Birth | Sex | Fasting |
|---|---|---|---|
| 46 v.o. | | Male | Yes |

| Patient Address | Additional Information |
|---|---|
| | |

| Date and Time Collected | Date Entered | Date and Time Reported | Physician Name | NPI | Physician ID |
|---|---|---|---|---|---|
| | | | | | |

Tests Ordered
**CBC, Platelet Ct, and Diff ; Liver Function (AST/ALT) ; Iron/Iron Binding Panel**

General Comments

| TESTS | RESULT | FLAG | UNITS | REFERENCE INTERVAL | LAB |
|---|---|---|---|---|---|
| **CBC, Platelet Ct, and Diff** | | | | | |
| Hematocrit | 40.6 | | % | 40.8 - 51.9 | 07 |
| Hemoglobin | 14.2 | | g/dL | 14.6 - 17.8 | 07 |
| Red Blood Cell Count | 8.4 | | x10E6/uL | 4.69 - 6.07 | 07 |
| White Blood Cell Count | 7.4 | | x10E3/uL | 3.2 - 10.6 | 07 |
| RDW | 13.0 | | % | 10.8 - 14.1 | 07 |
| MCV | 82.6 | | fL | 77.8 - 94.0 | 07 |
| MCH | 27.4 | | pg | 26.5 - 32.6 | 07 |
| MCHC | 34.4 | | g/dL | 32.7 - 36.9 | 07 |
| Imm.Granulocytes (Absolute) | 6.2 | | x10E3uL | 1.3 - 7.0 | 07 |
| Granulocytes (Percent) | 57.2 | | % | 44.0 - 76.0 | 07 |
| Eosinophils (Absolute) | 0.3 | | x10E3uL | 0.0 - 0.4 | 07 |
| Eosinophils (Percent) | 5.2 | | % | 0.0 - 6.0 | 07 |
| Basophil (Absolute) | 0.08 | | x10E3uL | 0.0 - 0.1 | 07 |
| Basophil (Percent) | 1.7 | | % | 0 - 2 | 07 |
| Monocytes (Absolute) | 0.64 | | x10E3uL | 0.2 - 0.7 | 07 |
| Monocytes (Percent) | 6.9 | | % | 4 - 8.9 | 07 |
| Lymphocytes (Absolute) | 2.26 | | x10E3uL | 0.8 - 3.1 | 07 |
| Lymphocytes (Percent) | 37.2 | | % | 15 - 43 | 07 |
| Platelets (Absolute) | 388 | | x10E3uL | 177 - 406 | 07 |
| **Liver Function** | | | | | |
| Aspartate aminotransferase (AST) | 223 | High | U/L | 8 - 43 | 07 |
| Alanine Aminotransferase (ALT) | 257 | High | U/L | 7 - 45 | 07 |
| **Iron and Total Iron Binding** | | | | | |
| Serum Iron | 2300 | Very High | ug/dL | 40 - 155 | 07 |
| Total Iron Binding Capacity | 328 | | ug/dL | 250 - 400 | 07 |
| Transferrin Saturation | 72 | Very High | % | 20 - 50 | 07 |

**PRELIMINARY INTERPRETATION:**

A preliminary diagnosis of Hemochromatosis can be made based on very high Serum Iron and Transferrin Saturation values. Elevated liver enzyme levels are indicative of liver failure which is a common consequence of Hemochromatosis.

There are well established pathogenic genetic variants that are strongly correlated with Hereditary hemochromatosis. The presence of a hereditary disorder has consequences for family members. Genetic testing and counseling should be considered by the patient and family members.